

THE TRACHOME

A. Definition

Trachoma is a contagious form of conjunctivitis caused by *Chlamydia trachomatis*, the leading cause of blindness worldwide.

B. Transmission mode

Transmission by direct or indirect contact.

Germ can be transmitted from one person to another in a number of ways:

- Children with trachoma have red, sticky eyes and sometimes runny noses. If the germ is in these discharges, it will be found **on fingers and linen** (paper towels, etc.). hands, clothes, bed linen, etc.).
- **Flies**: attracted by ocular discharge, they transmit the trachomatous germ.
- **Children sleeping next to each other**: the germ can easily be transmitted from one child to another through linen.

C. Clinical signs

- ✓ inflammation: redness, discharge, follicles and swelling of the membrane lining the inner surface of the eyelids
- ✓ scar formation on the inside of the eyelid (due to repeated infections). If the scars are significant, over time the eyelashes turn back towards the eyelid. inwards, giving rise to a condition known as **trichiasis** (the eyelashes rub against the eye, especially the cornea).

The constant rubbing of eyelashes against the cornea causes white scars to form, leading to reduced vision and blindness.

D. Classification according to the WHO

Five stages are defined in the WHO classification:

Stage 1: Follicular Trachoma or Follicular Trachomatous Inflammation (FTI)

Common in children.

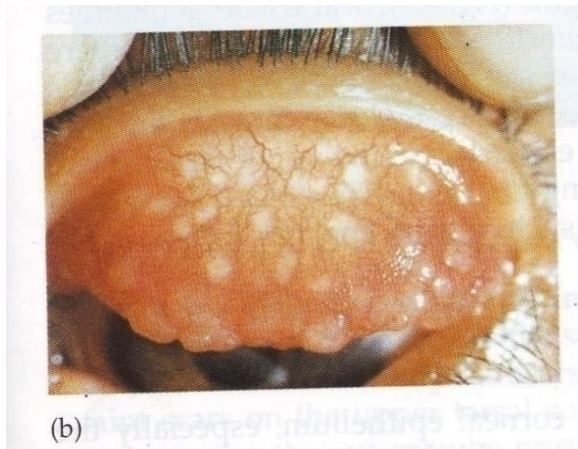
CLINICALLY we observe :

- ✓ red, sticky, itchy or sore eyes.

To be sure, we look at the inside of the upper eyelid (to do this, we have to turn the eyelid inside out).

- ✓ small white spots called follicles. This is known as TF or Follicular Trachomatous Inflammation (cases of progressive trachoma, on the outer surface of the eyelid).

Definition (according to WHO): *Presence at least five (5) follicles (0.5 mm or more in diameter) on the upper tarsal conjunctiva.*

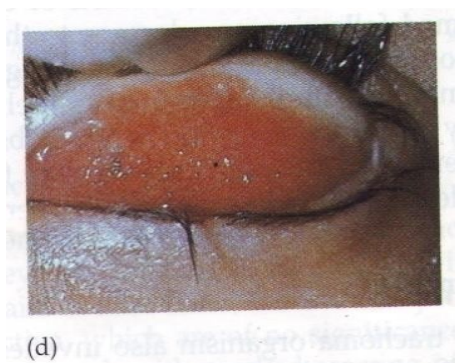


Type of lesion:

- Non-vascularized papules
- Slight inflammation
- Visible vessels

Stage 2: Inflammatory Trachoma or Intense Trachomatous Inflammation (ITI)

Definition (according to WHO): *Pronounced inflammatory thickening of the upper tarsal conjunctiva, masking more than half of the deep tarsal vessels.*



Types of lesions:

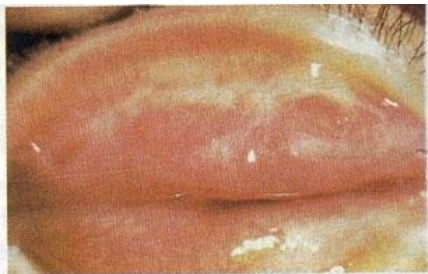
- Thickening of the tarsal conjunctiva
- Intense inflammation
- Large vessels are no longer visible

Stage 3: Trachoma Scar (TS)

The third sign (TS) is seen after repeated infections, when scars appear on the inside of the eyelid.

CLINICALLY, patients sometimes complain of having sand or insects in their eyes. Red or sticky eyes are not always observed, but if you look at the inside of the eyelid, you'll see traces of scarring, resembling whitish stripes. This is known as TS or trachomatous scarring.

Definition (according to WHO): *Presence of clearly visible scars in the tarsal conjunctiva.*



Type of lesions :

- scarring in the form of irregular, linear, whitish trabeculae.
- the conjunctiva is hypertrophied.
- Inflammation is subacute.
- Vessels are barely visible

Stage 4: Trachomatous trichiasis (TT)

Definition (according to WHO): *At least one eyelash floating against the eyeball, or traces of hair removal.*

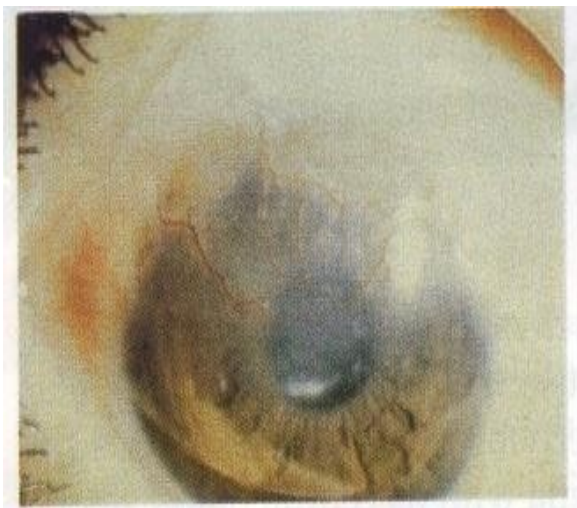


Types of lesions:

- adhesion of the conjunctiva to the cornea;
- the lashes are returned to the cornea and adhere.
- limited mobility of the cornea, reducing the eye's visual field. **Stage 5:**

Corneal opacity (whitish marks on the eye). **Definition (according to WHO):**

Corneal opacity clearly visible on the pupil.



Types of lesions:

- corneal opacity
- the upper part of the cornea is clearly opaque, while the lower part is transparent

E. Treatment

Treatment varies according to the stage of the disease:

- Stages 1 (TF) and 2 (TI):
 - Facial cleansing several times a day
 - Tetracycline ophthalmic 1%, 2 applications/day for 6 weeks
 - Oral azithromycin: - children: 20 mg/ Kg as a single dose;
- adult: 1g single dose

N.B. Whenever possible consider treating the whole family at the same time.

Table 1: Dosage of Azithromycin 200 mg/5ml oral suspension, according to size

Child's height (in cm)	Quantity of product to be administered (in ml)
62,7 - 68,2	04 ml
68,3 - 78,8	06 ml
78,9 - 89,4	08 ml
89,5 - 99,4	10 ml
99,5 - 111,1	12 ml
111,2 - 119,6	14 ml

Table 2: Dosage Azithromycin 250 tablets, according to size.

Individual size (in cm)	Quantity of product to be administered (in number of tablets)
94,7 - 122,3	2
122,4 - 141,5	3
>141,6	4

N.B. Whenever possible consider treating the whole family at the same time.

- Stage 3 (TS): no treatment (wait until stage 4 for surgical treatment)
- Stage 4 (TT): surgical treatment
- Stage 5 (CO): no treatment (irreversible stage leading to blindness).